

Meadows Mental Health Policy Institute

Response to Health and Human Services Department Request for Information (RIN 0955-AA13)

February 23, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy
Office of the National Coordinator
200 Independence Avenue, SW
Washington, DC 20201

RE: RFI Response – Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane:

The Meadows Mental Health Policy Institute (Meadows Institute) appreciates the opportunity to provide feedback on the use of artificial intelligence (AI) in behavioral health clinical care. The Meadows Institute recognizes the potential for AI to improve the quality, reach, and precision of behavioral health care, address severe workforce shortages, and help more people obtain the care they need. We are committed to ensuring the behavioral health field is not left out of health tech innovation, as has occurred in the past. We aim to leverage the positive benefits of AI while promoting the development and implementation of AI that is safe, accurate, and effective so that people with behavioral health conditions receive the care they need.

Independent and nonpartisan, the Meadows Institute works at the intersection of policy and practice to create equitable systemic change so that all people in Texas, the nation, and the world can obtain the health care they need. The Meadows Institute provides state and national leaders with data-driven, trusted policy and program guidance, and it works to shift the focus of new investments toward early intervention for children and families, address the behavioral health crisis in our jails and emergency rooms, and help all people with behavioral health needs recover and be well.

Company and product names referenced herein are provided solely for illustrative purposes and do not constitute an endorsement by the Meadows Institute.

Overview: Adopting AI in Behavioral Health Care

The Meadows Institute encourages ASTP/ONC to carefully consider behavioral health care as it develops approaches to regulation, reimbursement, and research and development to encourage AI adoption in clinical behavioral health care.

Positive Use Cases of AI in Behavioral Health

Nearly one in five adults in the United States lives with a mental illness, but only half of them receive behavioral health care.¹ Close to 90% of our nation's health care spending goes toward individuals with chronic and behavioral health conditions.^{2,3} When used to enhance – rather than replace – human judgment, AI holds immense potential to address long-standing challenges in the delivery of behavioral health care. AI's potential benefits extend across applications in behavioral health care, including workflow automation, provider training and quality monitoring, populations analytics, research and development, clinician support, patient engagement, and promotion of health between in-person visits. Application of AI in these areas may lead to increased access, improved quality, and reduced costs of behavioral health care.

A particularly beneficial potential application of AI in behavioral health care is supporting the evidence-based practice of measurement informed care (MIC). Starting with initial patient screening for behavioral health conditions, MIC involves repeated, systematic use of validated measures during clinical encounters to inform decision-making about treatment, thereby supporting – not replacing – clinical judgment. Providers and patients use measurement to help inform and develop an individualized treatment plan and continuously monitor and adjust treatment.

While simple conceptually, MIC can be cumbersome administratively if not optimally integrated into electronic health records (EHRs) and clinical workflows. Even when assessment data is collected and stored in an EHR, the data may not always be accessible in an easy-to-use format. This is particularly true for behavioral health data, as behavioral health has often seen underfunding and under-inclusion in EHR systems. These obstacles have served as a major impediment to large-scale adoption of MIC for behavioral health. Thus, despite the demonstrated benefits of MIC in behavioral health care, fewer than 20% of behavioral health providers use it.^{4,5,6} AI, and investments in technology more broadly, can help overcome many of the persistent barriers to clinician uptake of MIC by:⁷

- **Reducing administrative burden:** through automation of the collection and scoring of validated assessments, functional scores, and other data sources in real-time.
- **Easing Use by Clinicians:** by allowing for the visualization of data longitudinally and the detection of patterns, allowing clinicians to more easily identify trends and track progress, plateaus, or deterioration.
- **Expanding data sources:** by enabling the utilization of information from a broader array of sources, such as wearables and remote monitoring tools (e.g., sleep, activity), clinical

notes, voice analysis, and/or facial analysis, to provide objective measures and additional insights.

- **Improving treatment personalization:** by identifying key issues such as nonresponse to treatment or deterioration in functioning and allowing providers to adjust care plans earlier and more effectively.⁸

How data are measured, assessed, and used in behavioral health contexts is constantly evolving. AI holds the promise of being the next step in the evolution in measurement and monitoring of behavioral health care. Behavioral health conditions often have overlapping symptoms, and assessment and measurement rely heavily on subjective methods, which can, at times, make assessment and monitoring a challenge.

Further, behavioral health often lacks the objective biometrics found in other areas of health, such as A1C or cholesterol tests. While research is still needed to understand their appropriate application and generalizability, there is an emerging body of evidence to support the use of data that can be obtained from smartphones, wearables, and telehealth recordings, such as mobility, location, phone use, call log, or heart rate, (i.e., “digital phenotyping”) to provide more objective measures for behavioral health assessment. A recent systematic review found that physiological and behavioral data collected through digital phenotyping methods can be effectively used to detect and predict changes in symptoms of patients with behavioral health conditions, allowing for earlier intervention, in some cases even before an adverse event occurs.⁹ For example, the systematic review showed digital phenotyping methods were used to predict and detect relapse in schizophrenia to aid in reducing hospitalization, cost of care, and negative life impacts.¹⁰

Regulation

The accelerated adoption of AI in behavioral health care requires a careful balance between developing standards and safeguards to ensure transparency, trust, and standardization, while avoiding cumbersome regulations that stifle innovation. The Meadows Institute appreciates HHS’s efforts to minimize unnecessary regulatory burdens that drive up costs and hinder patient care. We also recognize the high degree of trust the public demands from behavioral health providers and the tools they use. To achieve this balance, we encourage HHS to focus its efforts on 1) facilitating the interoperability of behavioral health data, 2) strengthening privacy and data security, and (3) fostering transparency and trust in the use of AI in behavioral health care.

Interoperability: Interoperability and data standardization remain a challenge across the health care system, but the challenge is especially great for behavioral health. Behavioral health was largely left out of the Health Information Technology for Economic and Clinical Health (HITECH)

Act, which incentivized the rest of the health care system to build out its IT infrastructure over the last few decades. Consequently, behavioral health IT infrastructure and the funding needed to support it lag significantly behind physical health.

In addition to a lack of investment in EHR infrastructure, behavioral health lacks data standardization and terminology that is needed to support the exchange of health care data. This has exacerbated a cycle that hinders the behavioral health industry from leveraging and incorporating advances in the broader health IT ecosystem.

The threat of behavioral health missing out on another cycle of technological innovation is a very real and present danger. And as the pace of innovation increases with the advent of new AI-based tools, behavioral health may face the reality that it can never catch up again. Effective access to EHRs and to standardized, interoperable data are fundamental to ensuring that health systems and behavioral health providers can leverage AI innovation appropriately.

To ensure these goals are met, the Meadows Institute encourages ASTP/ONC to:

- Strongly support opportunities to incentivize the purchase and adoption of EHRs in behavioral health.
- Facilitate the inclusion of behavioral health data into existing EHR infrastructure to ensure integration of behavioral health data across health care settings.
- Expand the work of USCDI+BH and invest in pilot programs, such as the Behavioral Health Information Technology (BHIT) Pilot Program, which is working to develop standardized data components to be captured in the EHR that are unique and distinct to behavioral health.
- Work with Congress to identify pathways to greater adoption of EHRs in behavioral health. In June 2025, Congress passed H.R. 2483, the *SUPPORT for Patients and Communities Reauthorization Act of 2025*, with an amendment directing ONC to convene a roundtable on how best to increase adoption of EHRs in behavioral health and substance use services. HHS should utilize this as a turning point for behavioral health and do everything in its power to support the interoperability of behavioral health data to enable integration. We may not have another, better opportunity in the foreseeable future.

Privacy and data security: The privacy and data security of behavioral health data is of the utmost importance. But we cannot allow behavioral health to be overly “protected” to such an extent that innovation and investment become impossible. Heightened concerns about privacy cannot be used to justify excluding behavioral health from advancements, as we saw with the HITECH Act. Instead, regulations and standards must be in place to protect privacy and data security.

For instance, the Health Insurance Portability and Accountability Act (HIPAA) definition of “covered entities” does not adequately cover the range of new digital health and AI tools.¹¹ HHS should address this gap to ensure patient-facing tools and other AI applications that may connect to EHRs have the necessary privacy and cybersecurity requirements. Years of hard work to recover since the failure of the HITECH Act show us these goals need not be in conflict. The Meadows Institute looks forward to engaging with HHS and ASTP/ONC to address commonsense solutions that protect consumer privacy while avoiding the unnecessary obstacles to innovation and investment.

Trust and transparency: One of the biggest barriers to uptake of AI tools in behavioral health is provider and patient lack of trust in the safety and accuracy of AI tools.^{12,13} To foster trust and transparency in AI tools, it is critical for health systems and providers to understand how AI models are developed and tested. This will allow health systems to readily evaluate the aptness and safety of AI tools for their patient population and effectively communicate their decision-making process to stakeholders. This is already a challenging task for many health systems, especially in rural areas where necessary resources and expertise may be lacking. HHS should determine appropriate methods to require and/or incentivize AI companies to provide health systems with the necessary information to most readily make informed decisions about AI products to purchase and implement.¹⁴ Additionally, in order to promote patient understanding of when and how AI tools are used, health systems could be required to obtain informed consent.

AI Literacy: The continual and rapid pace of AI advancement necessitates a focus on AI literacy to ensure providers have the competence to critically interpret AI-generated measurements. Providers may need to “upskill” and acquire new and different skills in order to provide proper human oversight and interpretation of AI tools.¹⁵

Reimbursement

Technology-Enabled MIC and Value-Based Care

As discussed above, MIC is an essential practice for improving clinical care. Systematic measurement is also the foundation for the measurement of quality and fundamental for the incorporation of reimbursement methods based on the clinical outcomes achieved and/or the quality of the care provided (i.e., paying for value). For example, research shows that aggregated MIC data can be effectively used to assess whether a provider or a group of behavioral health providers is achieving a benchmark level of remission in their patients.

Because paying for value requires MIC, and AI and health information technology facilitate its implementation, value-based payment models can indirectly incentivize the use of AI in clinical

care. However, it is important that the incentives are sufficient to cover the cost of technology and human work involved in administering a robust MIC practice.

Technology-Enabled MIC and Remote Therapeutic Monitoring

Remote therapeutic monitoring (RTM) enables the use of new data from wearables and smartphones, such as sleep, physical activity, homework completion between therapy sessions, and medication adherence. These new data sources expand measurement and monitoring opportunities, broadening the potential of MIC in behavioral health care.

Currently, Medicare's RTM codes support payment for monitoring therapy adherence and therapy response of cognitive behavioral therapy. HHS should consider expanding the codes to cover other areas of behavioral health that can more broadly support the practice of MIC. Additionally, the Centers for Medicare and Medicaid Services (CMS) and the Center for Medicaid and CHIP Services (CMCS) should encourage the payment of RTM for behavioral health by issuing a bulletin to clarify opportunities to support MIC adoption through the payment of RTM.

Medicaid Reimbursement & Early Intervention

Interventions work best when administered close to symptom onset and before symptoms reach crisis, making early intervention a central goal of effective and cost-efficient care.^{16,17} AI has the potential to greatly facilitate this goal by supporting early intervention, screening, diagnosis, and monitoring of behavioral health conditions.

The childhood and adolescent period is a critical time for early intervention in behavioral health as nearly 50% of behavioral health conditions manifest by the age of 14.¹⁸ Because Medicaid is the single largest payer of behavioral health and insures almost 40% of all children in the U.S., it plays a critical role in the adoption of any new service or technology for children and adolescents. Thus, ensuring Medicaid covers the cost of AI technologies and other digital behavioral health tools is crucial. Medicaid can serve as both the engine and the proving ground for effectively deploying these technologies in support of early interventions, reducing costs and greatly increasing patient outcomes.

CMS and CMCS should issue an informational bulletin to clarify existing opportunities to support the adoption of AI and other digital behavioral health technologies. This should include opportunities to support MIC through value-based payment models and RTM codes, as mentioned above.

Research and Development

We encourage HHS to increase investment in programs that accelerate the development and adoption of AI-supported health innovation via public-private partnerships. ARPA-H has been one model of cross-sector partnership investment that can yield meaningful health benefits quickly. ARPA-H's current EVIDENT (Evidence-Based Validation & Innovation for Rapid Therapeutics in Behavioral Health) initiative, for example, has the potential to leverage public-private collaboration to bring precision care to behavioral health, narrowing the gap between advancements in physical and behavioral health.

HHS could also replicate other successful federally-led public-private AI demonstrations. In 2022, the Department of Veterans Affairs (VA) launched the Mission Daybreak grand challenge to transform suicide prevention through innovation and meet the unique needs of veterans. Initially awarding \$20 million to forty pilot demonstrations, Mission Daybreak remains a comprehensive program for accelerating prevention solutions through continuous awards for early-stage research, development and pilots.¹⁹

RFI Questions

Q5: How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS could create a program to test and validate behavioral health AI systems focusing on accuracy, safety, fairness, and, most importantly, the demonstration of clinically meaningful behavioral health outcomes. HHS can and should demand impact and efficacy in any validation effort. This should include supporting common evaluation frameworks and shared behavioral health outcomes to ensure AI performance and real-world impact can be assessed consistently and routinely across contexts and populations.

HHS should also continue to support the development and dissemination of USCDI+BH interoperability standards to facilitate the exchange of behavioral health data via AI apps, EHRs, and other systems. HHS should also address data management standards for behavioral health, focusing on how these data should be safeguarded and protected as it is exchanged and whether current HIPAA standards are adequate to meet these evolving needs.

Q6: Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

As discussed in the sections on regulation and reimbursement above, AI used in support of the implementation of MIC is a high-impact, established use case of AI in behavioral health care.

Additional high-impact use cases include:

1. Optimizing crisis response through optimized training and risk prediction

Crisis hotlines, text lines, and 24/7 online chats, such as the National Alliance on Mental Illness (NAMI), the National Suicide Prevention Lifeline, and Veterans Crisis Line, are a key component of the crisis response system to prevent suicide.^{20,21} AI is already helping these critical, high-stakes services provide faster, more effective, and more personalized care.

One challenge crisis platforms face, whether call-, chat-, or text-based, is managing the sheer volume of calls. For instance, NAMI experienced a 60% increase in help-seekers between 2019 and 2021. Despite efforts to hire and train large numbers of staff, many crisis platforms are still unable to meet demand, and many seeking care go without it. To address this issue, researchers at Stanford University's Human-Centered Artificial Intelligence (HAI) center developed a machine learning (ML) system based on natural language processing, called Crisis Message Detector 1 (CMD-1), to identify high-risk messages, triage messages, and help those in most imminent need receive support faster. One recent study showed that deployment of CMD-1 reduced daily median wait times for crisis calls, chats, and texts from nine hours to an average of eight to 13 minutes.²²

Another challenge facing crisis call centers and other crisis platforms is the need for skilled, empathic staff to respond to a wide range of high-stakes scenarios. AI can potentially be deployed to efficiently and effectively train crisis response workers to be ready as soon as they enter the workforce, and to provide continuous quality monitoring of services.²³ A high-impact use case is the VA's partnership with ReflexAI to provide specialized training and practical skills for Veterans Crisis Line responders through simulated training. ReflexAI uses AI to create simulations of veteran personas to create realistic roleplays and give feedback, providing consistency, frequency, and quality not previously possible in training programs.²⁴ Critically, ReflexAI does not replace human care but bolsters it.

2. Supporting evidence-based integrated care

Evidence-based integrated care models, which treat co-occurring physical and behavioral health needs in the same location, have been proven to dramatically improve outcomes and reduce health care costs. Models such as the psychiatric Collaborative Care Model, which is targeted to serve individuals with mild to moderate needs, enhances early detection, treatment, and relapse prevention.²⁵

Unfortunately, implementation of integrated care is relatively low given its proven effectiveness, often owing to administrative challenges.²⁶ However, AI and other technology

can help automate administrative tasks and support team efficiency to relieve many of these burdens.²⁷ For instance, AI can potentially help capture eligible services for time-based integrated care models and reduce revenue uncertainty, a major reason practices report either abandoning or never starting an integrated care model. Additionally, AI can support case review efficiency through automated case synthesis, which summarizes symptoms, scores, medication trials, adherence, and key events. MIC is a critical component of effective CoCM implementation, so the opportunities to support MIC through AI hold promise for improving CoCM uptake and ongoing use.

Emerging research demonstrates the value of AI and technology more broadly in implementing integrated care models efficiently and effectively. For instance, NeuroFlow is a technology platform that can be deployed to support the implementation of integrated care. It uses patient engagement tools, AI, and health data to monitor, track, and manage patient behavioral health risks. In one study of integrated care implemented in primary care and OBGYN settings, the use of NeuroFlow's platform significantly improved behavioral health outcomes and reduced emergency department (ED) utilization compared to a control group receiving treatment as usual (TAU).²⁸ Because ED use reducing preventable ED utilization can reduce the total cost of care.²⁹

Q8: Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Many chronic physical illnesses, including diabetes, cardiovascular disease, and chronic pain, co-occur alongside behavioral health conditions. As discussed above, integrated care models have been shown to improve both mental and physical health outcomes for people with co-morbid chronic diseases.³⁰ AI has the potential to help extract, synthesize and analyze data across health care settings to provide a more holistic picture of patients in support of the delivery of integrated care. Because data fragmentation limits the potential of AI in supporting integrated care, greater interoperability is necessary to leverage AI more fully.

As discussed above, behavioral health is lagging in health information technology (HIT) infrastructure. Of the 1,059 inpatient psychiatric hospitals in the U.S., only 47% have adopted EHR technology.³¹ There are similar challenges in the outpatient setting, as EHR and HIT implementation may be cost-prohibitive for many smaller clinics and private practices. To successfully deploy AI in behavioral health care, it is critical to address behavioral health EHR infrastructure, data standardization, and interoperability.

Q9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

AI shows strong promise in addressing the challenges of a burdened behavioral health workforce and the slow uptake of behavioral health integration. AI tools can improve behavioral health provider training and quality monitoring, while reducing administrative load. Clinicians, as well as non-licensed behavioral health professionals, can automate their workflows with AI tools that can transcribe and summarize session notes, automate intake data, and support administrative tasks. Patient-facing AI-enabled tools integrated into clinical care can increase patients' access to proven evidence-based practices (EBPs). For example, they can be used to increase engagement with homework between sessions (an integral component of many EBPs) as well as with accepted therapy and interventions generally.³² These digital behavioral health tools can be layered on top of sessions with a clinician to extend the therapeutic process or, in some cases, to reduce the amount of time or frequency of sessions.³³ Patients and caregivers may also struggle to find the best provider to meet their needs. AI-enabled data analytics can be used to more effectively and efficiently match patients' needs with provider expertise, insurance, and geographic fit.³⁴

Patients and caregivers also frequently report being frustrated by the lack of communication and coordination across health care settings and the need to reshare medical history repeatedly.³⁵ In addition to frustrating patients, this lack of coordination can prove to be a barrier to care itself; however, AI that supports interoperability can help overcome this persistent challenge in behavioral health care by facilitating the sharing of data across providers and health settings, thereby limiting the need for patients to repeatedly provide their history to each provider or practice.

At the same time, patients and caregivers have justifiable concerns with the use of AI, including:³⁶

- Privacy and security of patient data, particularly biometric data. Data and personal information disclosed as part of therapy discussions and recorded in clinical notes can be highly sensitive.
- Fear of losing access to human providers. Research shows that patients and their caregivers want to ensure their clinicians, and not AI, are ultimately guiding them through treatment and making care decisions. The therapeutic alliance – or the positive relationship between behavioral health provider and patient – must be preserved.
- Safety and accuracy of AI models. It is important for HHS, health care systems, and AI developers to build trust around their products by showing accurate and effective solutions across different populations.

Q10: Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

While there is a growing and significant body of research demonstrating the positive impact of many AI tools in behavioral health care, there is a lack of trust and consensus that AI will be beneficial to all patients in real-world settings. A major factor contributing to this lack of trust is that many research studies are based on small datasets that lack clinical validation and may not accurately reflect diagnosed populations. A recent review of behavioral health datasets used in AI development highlighted several challenges, such as:³⁷

- Small datasets lead to poor reliability and generalizability. Lack of variation in geography, language, and clinical setting intensify this issue.
- Lack of standardization impedes training and consistent evaluation.
- Inadequate scalable privacy mechanisms lead to isolation of most datasets.

A foundational step HHS could take to accelerate the adoption of AI in behavioral health care is to ensure ARPA-H and the National Institute for Mental Health (NIMH) work with the private sector to help develop larger, more varied, and clinically valuable datasets and to further establish standardized methodologies for data collection and utilization. This foundation will enable digital behavioral health innovators to access representative datasets to develop the most effective, reliable, and trustworthy products. One such model, also mentioned above, is the ARPA-H EVIDENT program, which solicited longitudinal, de-identified data collected from registered clinical trials that will be curated, stored, and used in future research to assess effectiveness of therapeutic modalities for specific patients.³⁸

The agentic nature of AI poses another challenge to its adoption. Traditional assessment and validation of interventions and medical devices is based on static interventions. In contrast, AI applications are continually learning and adapting over time. Thus, AI clinical tools require ongoing performance evaluation, not just one-time approvals. More needs to be understood about how to best assess AI clinical tools to detect drift in performance over time. Significant federal investment in independent, longitudinal research to understand the long-term impacts of AI on clinical care quality and mental health outcomes is needed.

Furthermore, to ensure trust in the adoption of AI in behavioral health care, HHS should support research on the most robust privacy practices and patient and caregiver preferences for use and consent of AI in behavioral health. Companies are increasingly moving to an “opt out” system for the use of AI in routine care. The impact on patient trust and engagement should be assessed as new practices and procedures are adopted.

Conclusion

The Meadows Institute looks forward to working with you to ensure that behavioral health care, and people with behavioral health conditions, benefit from AI innovation, while promoting its responsible use. Please contact John Snook, Chief Policy Officer, at jsnook@mmhpi.org or (571) 331-5725 with any questions.

Sincerely,

A handwritten signature in black ink, appearing to read "John Snook".

John Snook

Chief Policy Officer, Meadows Mental Health Policy Institute

References

- ¹ Steadman, H. J., Osher, F. C., Robbins, P. C., Case, B., & Samuels, S. (2009). Prevalence of Serious Mental Illness Among Jail Inmates. *Psychiatric Services*, 60(6), 761–765. <https://doi.org/10.1176/ps.2009.60.6.761>
- ² CDC. (2025, April 29). *Mental Health*. Mental Health. <https://www.cdc.gov/mental-health/index.html>
- ³ *Establishing the President's Make America Healthy Again Commission*. (2025, February 13). The White House. <https://www.whitehouse.gov/presidential-actions/2025/02/establishing-the-presidents-make-america-healthy-again-commission/>
- ⁴ Fortney, John, Rebecca Sladek, & Jürgen Unützer. (2015). *Fixing behavioral health care in America: A national call for integrating & coordinating specialty behavioral health care with the medical system*. The Kennedy Forum. https://www.patrickjkenedy.net/app/uploads/2020/05/KennedyForum-BehavioralHealth_FINAL_3.pdf
- ⁵ Lewis, C. C., Boyd, M., Puspitasari, A., Navarro, E., Howard, J., Kassab, H., Hoffman, M., Scott, K., Lyon, A., Douglas, S., Simon, G., & Kroenke, K. (2019). Implementing measurement-based care in behavioral health: A review. *JAMA Psychiatry*, 76(3), 324. <https://doi.org/10.1001/jamapsychiatry.2018.3329>
- ⁶ Lewis, C. C., Boyd, M., Puspitasari, A., Navarro, E., Howard, J., Kassab, H., Hoffman, M., Scott, K., Lyon, A., Douglas, S., Simon, G., & Kroenke, K. (2019). Implementing Measurement-Based Care in Behavioral Health: A Review. *JAMA Psychiatry*, 76(3), 324. <https://doi.org/10.1001/jamapsychiatry.2018.3329>
- ⁷ Lewis, C. C., Boyd, M., Puspitasari, A., Navarro, E., Howard, J., Kassab, H., Hoffman, M., Scott, K., Lyon, A., Douglas, S., Simon, G., & Kroenke, K. (2019). Implementing Measurement-Based Care in Behavioral Health: A Review. *JAMA Psychiatry*, 76(3), 324. <https://doi.org/10.1001/jamapsychiatry.2018.3329>
- ⁸ Walfish, S., McAlister, B., O'Donnell, P., & Lambert, M. J. (2012). An Investigation of Self-Assessment Bias in Mental Health Providers. *Psychological Reports*, 110(2), 639–644. <https://doi.org/10.2466/02.07.17.PRO.110.2.639-644>
- ⁹ Bufano, P., Laurino, M., Said, S., Tognetti, A., & Menicucci, D. (2023). Digital Phenotyping for Monitoring Mental Disorders: Systematic Review. *Journal of Medical Internet Research*, 25, e46778. <https://doi.org/10.2196/46778>
- ¹⁰ Bufano, P., Laurino, M., Said, S., Tognetti, A., & Menicucci, D. (2023). Digital Phenotyping for Monitoring Mental Disorders: Systematic Review. *Journal of Medical Internet Research*, 25, e46778. <https://doi.org/10.2196/46778>
- ¹¹ Bari, L., & O'Neill, D. P. (n.d.). Rethinking patient data privacy in the era of digital health. *Health Affairs Forefront*. <https://doi.org/10.1377/forefront.20191210.216658>
- ¹² Higgins, O., Short, B. L., Chalup, S. K., & Wilson, R. L. (2023). Artificial intelligence (AI) and machine learning (ML) based decision support systems in mental health: An integrative review. *International Journal of Mental Health Nursing*, 32(4), 966–978. <https://doi.org/10.1111/inm.13114>
- ¹³ Cross, S., Bell, I., Nicholas, J., Valentine, L., Mangelsdorf, S., Baker, S., Titov, N., & Alvarez-Jimenez, M. (2024). Use of AI in Mental Health Care: Community and Mental Health Professionals Survey. *JMIR Mental Health*, 11(1), e60589. <https://doi.org/10.2196/60589>
- ¹⁴ Ross, C. (2025, December 23). Rolling back health AI transparency rule will shift the burden of vetting to health systems. *STAT*. <https://www.statnews.com/2025/12/23/deregulating-health-ai-transparency-rule-shifts-burden/>
- ¹⁵ *Upskilling the Health Information Workforce in the Age of AI*. (n.d.). The American Health Information Management Association (AHIMA). Retrieved April 21, 2025, from <https://www.ahima.org/education-events/artificial-intelligence/upskilling-the-health-information-workforce-in-the-age-of-ai/>
- ¹⁶ Membride, H. (2016). Mental health: Early intervention and prevention in children and young people. *British Journal of Nursing*, 25(10), 552–557. <https://doi.org/10.12968/bjon.2016.25.10.552>
- ¹⁷ Kane, J. M., Robinson, D. G., Schooler, N. R., Mueser, K. T., Penn, D. L., Rosenheck, R. A., Addington, J., Brunette, M. F., Correll, C. U., Estroff, S. E., Marcy, P., Robinson, J., Meyer-Kalos, P. S., Gottlieb, J. D., Glynn, S. M., Lynde, D. W., Pipes, R., Kurian, B. T., Miller, A. L., Azrin, S. T., ... Heinssen, R. K. (2016). Comprehensive versus usual community care for first episode psychosis: 2-year outcomes from the NIMH RAISE early treatment program. *American Journal of Psychiatry*, 173(4), 362–372. <https://doi.org/10.1176/appi.ajp.2015.15050632>
- ¹⁸ Kessler, R. C., Angermeyer, M., Anthony, J. C., DE Graaf, R., Demyttenaere, K., Gasquet, I., DE Girolamo, G., Gluzman, S., Gureje, O., Haro, J. M., Kawakami, N., Karam, A., Levinson, D., Medina Mora, M. E., Oakley Browne, M. A., Posada-Villa, J., Stein, D. J., Adley Tsang, C. H., Aguilar-Gaxiola, S., ... Ustün, T. B. (2007). Lifetime prevalence and

age-of-onset distributions of mental disorders in the World Health Organization's World Mental Health Survey Initiative. *World Psychiatry: Official Journal of the World Psychiatric Association (WPA)*, 6(3), 168–176.

¹⁹ About Mission Daybreak. (n.d.). *Mission Daybreak*. Retrieved February 4, 2026, from

<https://www.missiondaybreak.net/about-mission-daybreak/>

²⁰ *Using NLP to Detect Mental Health Crises | Stanford HAI*. (2024, January 8).

<https://hai.stanford.edu/news/using-nlp-detect-mental-health-crises>

²¹ *AI technology is helping crisis line responders—VA News*. (2024, September 2). <https://news.va.gov/133911/ai-technology-is-helping-crisis-line-responders/>

²² Swaminathan, A., López, I., Mar, R. A. G., Heist, T., McClintock, T., Caoili, K., Grace, M., Rubashkin, M., Boggs, M. N., Chen, J. H., Gevaert, O., Mou, D., & Nock, M. K. (2023). Natural language processing system for rapid detection and intervention of mental health crisis chat messages. *NPJ Digital Medicine*, 6, 213.

<https://doi.org/10.1038/s41746-023-00951-3>

²³ Kacie Kelly, Sam Dorison, Emily Feinstein, & Tommy Marzella. (n.d.). *When AI Saves Lives: Real Applications in Mental Health*. Retrieved July 8, 2025, from <https://schedule.sxsw.com/2025/events/PP153713>

²⁴ U.S. Department of Veterans Affairs. (n.d.). Mission Daybreak: ReflexAI. *Mission Daybreak*.

<https://www.missiondaybreak.net/2024-team-spotlights/reflexai/>

²⁵ Meadows Mental Health Policy Institute. (2023). *Collaborative Care Implementation Costs Across Ten United States Health Systems*.

²⁶ The Comprehensive Health Integration (CHI) Framework. (2025, February 13). *National Council for Mental Wellbeing*. <https://www.thenationalcouncil.org/resources/the-comprehensive-health-integration-framework/>

²⁷ Pardes, A., Rene, R., Chun, P., & Cherson, M. (2022). Technology-enabled Behavioral Health Integration Decreases Emergency Department Utilization. *Clinical Practice & Epidemiology in Mental Health*, 18(1), e174501792208150. <https://doi.org/10.2174/17450179-v18-e2208150>

²⁸ Pardes, A., Rene, R., Chun, P., & Cherson, M. (2022). Technology-enabled Behavioral Health Integration Decreases Emergency Department Utilization. *Clinical Practice & Epidemiology in Mental Health*, 18(1), e174501792208150. <https://doi.org/10.2174/17450179-v18-e2208150>

²⁹ Pardes, A., Rene, R., Chun, P., & Cherson, M. (2022). Technology-enabled Behavioral Health Integration Decreases Emergency Department Utilization. *Clinical Practice & Epidemiology in Mental Health*, 18(1), e174501792208150. <https://doi.org/10.2174/17450179-v18-e2208150>

³⁰ Hellstern, R. B., Lamson, A. L., Jensen, J. F., Martin, M. P., & Hylock, R. H. (2025). Physical and mental health outcomes of integrated care: Systematic review of study. *Families, Systems, & Health*.

<https://doi.org/10.1037/fsh0000960>

³¹ Hu, X., Qu, H., Houser, S. H., Chen, H., Zhou, J., & Yu, M. (2020). Hospital characteristics associated with certified ehr adoption among US psychiatric hospitals. *Risk Management and Healthcare Policy*, Volume 13, 295–301.

<https://doi.org/10.2147/RMHP.S241553>

³² Figueroa, C. A., & Aguilera, A. (2020). The Need for a Mental Health Technology Revolution in the COVID-19 Pandemic. *Frontiers in Psychiatry*, 11, 523. <https://doi.org/10.3389/fpsy.2020.00523>

³³ Weiner, S. (2022, August 9). *A growing psychiatrist shortage and an enormous demand for mental health services*. AAMC.

³⁴ Lee, C., Mohebbi, M., O'Callaghan, E., & Winsberg, M. (2024). Large Language Models Versus Expert Clinicians in Crisis Prediction Among Telemental Health Patients: Comparative Study. *JMIR Mental Health*, 11(1), e58129.

<https://doi.org/10.2196/58129>

³⁵ Eastman, M. R., Kalesnikava, V. A., & Mezuk, B. (2022). Experiences of care coordination among older adults in the United States: Evidence from the Health and Retirement Study. *Patient Education and Counseling*, 105(7), 2429–2435. <https://doi.org/10.1016/j.pec.2022.03.015>

³⁶ Benda, N., Desai, P., Reza, Z., Zheng, A., Kumar, S., Harkins, S., Hermann, A., Zhang, Y., Joly, R., Kim, J., Pathak, J., & Reading Turchioe, M. (2024). Patient Perspectives on AI for Mental Health Care: Cross-Sectional Survey Study. *JMIR Mental Health*, 11, e58462. <https://doi.org/10.2196/58462>

³⁷ Mandal, A., Adhikary, P. K., Arnaout, H., Gurevych, I., & Chakraborty, T. (2025). *A Comprehensive Review of Datasets for Clinical Mental Health AI Systems (Version 2)*. arXiv. <https://doi.org/10.48550/ARXIV.2508.09809>

³⁸ Hollowell, A. (2025, November 24). HHS's ARPA-H Agency Commits \$100M to Accelerate Personalized Mental Health Interventions. *Behavioral Health Business*. <https://bhbusiness.com/2025/11/24/hhss-arpa-h-agency-commits-100m-to-accelerate-personalized-mental-health-interventions/>